

Pain Assessment and Management

Policy Number	POL-PC-062	Owner / Department	Nursing / Pharmacy
Effective Date	2023-05-01	Version	2.1
Last Revised	2026-03-04	Next Review	2028-03-04
Approved By	Patient Safety & Quality Committee	Applies To	All patients in all inpatient, observation, emergency, and procedural settings

1. Purpose

To assess and manage pain safely and effectively through individualized, multimodal treatment and timely reassessment, while monitoring for opioid-induced sedation and respiratory depression, consistent with The Joint Commission pain-assessment and pain-management standards.

2. Scope

Applies to all patients. Pain is screened on admission and assessed with an age- and condition-appropriate validated tool, reassessed after every intervention, and documented.

3. Definitions

- **Multimodal analgesia** — combining non-opioid medications, non-pharmacologic methods, and opioids when indicated to control pain while minimizing opioid dose.
- **Sedation scale** — a validated scale (e.g., the Pasero Opioid-Induced Sedation Scale, POSS) used to detect advancing sedation before respiratory depression.
- **Opioid-induced respiratory depression** — hypoventilation and hypoxia resulting from opioid effect, a potentially fatal adverse event.

4. Reassessment Timeframes by Route

Reassess pain, sedation level, and respiratory status after each intervention within the timeframe matched to the expected onset of the route used. Reassess sooner for any sign of oversedation.

Route / intervention	Reassess within
Intravenous (IV) push opioid	15–30 minutes
Subcutaneous / intramuscular	30 minutes
Oral / enteral analgesic	60 minutes
Epidural / patient-controlled analgesia (PCA)	Per protocol; sedation & respirations hourly × the first 12 hours
Non-pharmacologic measure	60 minutes, or per the intervention

5. Opioid Safety Monitoring

IMPORTANT — Advancing sedation precedes opioid-induced respiratory depression. If a patient is difficult to arouse, has a declining sedation score, or shows a respiratory rate below 8 or oxygen desaturation, hold further opioids, stimulate and support ventilation, administer naloxone per protocol, and call for rapid response immediately.

- Monitor sedation level and respiratory rate (and capnography/oximetry where indicated) with each opioid dose and during peak effect, more intensively for opioid-naïve, post-operative, and sleep-apnea patients.
- Naloxone and reversal supplies are immediately available wherever opioids are administered.
- Avoid stacking opioids and concurrent sedatives without reassessment between doses.

6. Severity & Reporting

Events are classified and reported under POL-RM-013. Opioid-induced respiratory depression requiring naloxone, rapid response, or rescue is at least SEV-2 (SEV-1 if it results in severe harm, permanent harm, or death) — notify the attending provider and Risk Management, and for SEV-1 the Nursing Supervisor and Administrator-on-Call, within 1 hour. An under-treated-pain complaint or a late reassessment without harm is SEV-3 or a near miss, reported within 24 hours. Medication errors are additionally reported under the medication-error policy.

7. Documentation Requirements

- Pain score with the tool used; location, quality, and acceptable comfort/function goal.
- Intervention given (drug, dose, route) and non-pharmacologic measures.
- Reassessment time and result, including sedation score and respiratory status.
- Any adverse effect, reversal given, and provider notification.

8. References

- The Joint Commission, pain-assessment and pain-management standards (Provision of Care).
- Pasero Opioid-Induced Sedation Scale (POSS).
- CDC Clinical Practice Guideline for Prescribing Opioids for Pain.
- Related: POL-PS-003 (medication errors), POL-RM-013 (severity & reporting).